

Executive Summary

- **CMS Clarification (ASP):** In the CY 2026 Physician Fee Schedule final rule, CMS explicitly “clarifies” that units of selected drugs sold at the maximum fair price (MFP) are included in the calculation of the manufacturer’s ASP, effective January 1, 2026 ¹. This means manufacturers must count all Part B sales at or below the negotiated MFP in both the ASP numerator (net sales) and denominator (units).
- **Part B Payment Change:** Starting in the first negotiation payment year 2028, Medicare Part B will pay **106% of the MFP** for selected drugs (instead of ASP+6%) ². For the interim years 2026–2027, drugs with negotiated prices but Part B use still follow ASP-based payment unless the drug is formally renegotiated in cycle 2 ³.
- **Statutory Basis:** Section 1847A(c) of the Social Security Act defines ASP as total sales (minus certain excluded sales) divided by units ⁴. The statute does *not* exempt negotiated/MFP sales from ASP calculations, so CMS used its regulatory authority to confirm they must be included. No separate exclusion for MFP exists in statute, so by default these sales count towards ASP unless CMS says otherwise.
- **Reporting Mechanics:** Manufacturers should treat MFP sales like any other sales: report the actual revenue (after the MFP rebate) and corresponding units in the quarterly ASP submission. In practice, if a product’s negotiated MFP is \$X and Y units are sold at that price in a quarter, then report \$X×Y in sales and Y in units. CMS has not issued special alternative reporting rules for MFP, implying the standard ASP-data submission (per 42 CFR § 414.902–§ 414.904) applies. For example, if 1,000 units sold at MFP \$100 each and 2,000 units sold at other prices in Q1, $ASP = (1,000 \times 100 + \text{revenue from other 2,000 units}) / 3,000$. Including MFP units typically *lowers* the weighted-average ASP (assuming $MFP < \text{list price}$).
- **Payment Impact:** Once a Part B–covered drug enters its MFP applicability period (first in 2028), CMS pays 106% of the MFP instead of ASP+6% ². During those quarters, the official Part B Drug Payment Limit file will display the MFP-based payment limit ¹. Prior to 2028, or for drugs not yet in their MFP period, payment remains ASP-based. Thus, after 2028 Medicare payment decouples from ASP for those drugs, though ASP reporting continues for all other programs and calculations.
- **Downstream Effects:** Because including low-priced MFP sales tends to reduce reported ASP, this can affect downstream metrics. A lower ASP widens the spread to WAC or AWP, which may raise Part B beneficiary coinsurance (6% of a smaller ASP) and affect state and commercial reimbursements that reference ASP. It also could alter calculations of Medicaid best price or 340B ceiling price, though CMS has not explicitly addressed whether MFP rebates count as Medicaid best price or are excluded from 340B (likely not counted as sales to *non-profit* purchasers). In Part D and commercial contracts that benchmark to ASP or use ASP-based formulas, changes in ASP from MFP may have side effects. Manufacturers must also watch compliance risk: CMS can audit ASP submissions and has warned that misclassifying MFP transactions (e.g. trying to exclude them) would violate reporting rules ¹.
- **Open Issues & Recommendations:** CMS guidance is clear on inclusion of MFP units, but some practical questions remain. For instance, if a manufacturer sells above MFP and later issues a retroactive rebate (to meet MFP), should the initial sale be reported as at full price with a separate price concession? CMS has not detailed that scenario; manufacturers may need to treat such rebates as price concessions (deducted in ASP) or net the sale to MFP. We recommend consulting CMS (via HPMS or enforcement channels) if needed. Companies should update their ASP-reporting systems to handle MFP sales and maintain documentation of their methodology (CMS now requires explanation

of “reasonable assumptions” in quarterly submissions ⁵ ¹). They should also monitor how CMS’s Part B Payment Limit files change (the new MFP-based rates will appear once applicable) and plan for beneficiary/co-pay impacts.

Timeline (Key Dates)

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timeline
    title MFP-ASP Implementation Timeline
    2022 : Inflation Reduction Act (IRA) is enacted, authorizing drug price negotiation
    2025 : Oct 31 - CMS finalizes CY2026 PFS rule (effective Jan 1, 2026) clarifying MFP inclusion in ASP 1
    2026 : Jan - MFP units counted in ASP; drug price negotiation (initial cycle) prices set for 2026-27
    2028 : Jan - Part B payment transitions to 106% of MFP for selected drugs 2 (beginning first “price applicability year”)
  
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CMS Statements on MFP and ASP

The table below compares CMS’s official statements about MFP treatment across key documents:

Source (Date)	Quote / Policy
CY2026 PFS Final Rule Fact Sheet (Oct 2025)	<i>“We are clarifying in this final rule that units of selected drugs sold at the maximum fair price (MFP) are included in the calculation of the manufacturer’s ASP described in section 1847A(c) of the... Act effective January 1, 2026.”</i> ¹
CY2026 PFS Proposed Rule Fact Sheet (Jul 2025)	<i>“We are clarifying in this proposed rule that units of selected drugs sold at the maximum fair price are included in the calculation of the manufacturer’s ASP described in section 1847A(c) of the... Act effective January 1, 2026.”</i> ⁶
Part B Drug Payment Limits Overview (Mar 2026)	<i>“Beginning in the initial price applicability year 2028, for selected drugs payable under Part B, section 1847A(b)(1)(B) of the Act sets the Medicare Part B payment limit...as 106 percent of the MFP.”</i> ² <i>“For selected drugs having a negotiated price during the initial price applicability year (2026–2027)... the payment limit will not be based on the MFP unless selected for renegotiation...”</i> ³
CMS Part B ASP FAQs (Mar 2026)	Discusses quarterly ASP reporting and payment limit publication; does not exclude MFP sales from ASP reporting, implying inclusion by default. (Does not explicitly mention MFP.)
CMS Guidance (Sept 2024) – Negotiation	No explicit mention of ASP calculation; guidance focuses on negotiation processes. (CMS has not issued separate ASP-reporting rules for MFP beyond the PFS rule.)

Statutory and Regulatory Basis

Under **42 U.S.C. §1395w-3a(c)**, a manufacturer's ASP is defined as "*the manufacturer's sales to all purchasers*" (minus statutorily excluded sales) divided by "*the total number of such units*" sold ⁴. The statute explicitly excludes only: (1) sales already exempt from Medicaid best price (Statute §1927(c)(1)(C)(i)), and (2) nominal sales ⁷. **No provision** in section 1847A(c) or elsewhere exempts negotiated/MFP sales from ASP. In fact, §1847A(c)(3) lists the types of price concessions to *include* (volume discounts, rebates, etc.) and the MFP is not among any excluded category ⁸. Thus, by default all sales (including those at MFP) must be reported.

CMS cited its authority under the PFS and the Medicare program to "clarify" this in rulemaking. Section 1847A(c)(5)(C) even authorizes CMS to implement ASP policies by regulation or instruction. In the IRA's drug negotiation title (PHS Act §§1191–1198), Congress did not instruct any special ASP exclusion for negotiated prices. Congress did instruct (in §1194(b)(1)(B) of the Social Security Act) that "the payment limit during the price applicability period [is] 106 percent of the MFP" for Part B drugs ², which CMS adopted in guidance ². Outside of payment, the law left ASP reporting unchanged, so CMS interpreted the existing ASP law to mean MFP transactions count in ASP.

Reporting Mechanics

What Manufacturers Should Report: CMS's clarification means quarterly ASP reports should include MFP transactions **just like any other sales**. In practice, manufacturers should:

- **Include MFP Units in Denominator:** Report all units of a selected drug that were dispensed (or sold) to all purchasers, including any sold at the negotiated price.
- **Include Net Revenue in Numerator:** Report the actual net sales revenue from those units. If the manufacturer sells at the MFP (or sells at higher WAC but then pays an MFP rebate), the net per-unit revenue effectively equals the MFP. That net amount is added to the total sales.
- **Apply Standard Deductions:** Deduct any allowed price concessions (rebates, discounts, chargebacks) *per 42 U.S.C. 1395w-3a(c)(3)*. Negotiated rebates are not Medicaid rebates, so CMS did not label them as deductible "rebates under section 1927." (CMS guidance did not explicitly categorize the MFP transaction; however, by instructing inclusion of units, CMS effectively treats the MFP as the sale price.)

For example, suppose a new Part B drug has negotiated MFP \$150/unit starting Q1 2026. In Q1, the manufacturer's records show: - 800 units sold at \$200 (list price) to various providers, and - 200 units sold under the MTF contract at \$150 (MFP).

The manufacturer's Q1 ASP would be calculated as:

$$ASP_{Q1} = \frac{(800 \times 200) + (200 \times 150)}{800 + 200} = \frac{(160,000 + 30,000)}{1000} = \$190.$$

If CMS had excluded the 200 MFP units, the ASP would be $\$(800 \times 200) / 800 = \200 , which CMS has now ruled is not correct.

Reporting Process: ASP data are submitted by NDC/quarter via CMS's ASP Data Collection system (HPMS). CMS's existing instructions (42 CFR § 414.902–.904) require submission of “sales by all purchasers” each quarter. There is no new HPMS module for negotiation: manufacturers should simply code those sales under the product's NDC as they normally would. CMS's ASP FAQs (updated March 2026) do not mention negotiation, indicating no separate form or special coding – report MFP sales as part of total sales. CMS does require (starting 2026) a “reasonable assumption” letter explaining how any assumptions were made, including MFP units ⁵, but that is documentation, not a change in the numbers reported.

Quarterly Timeline: Manufacturers must continue to report quarterly ASP (within 30 days of quarter-end). For example, Q1 2026 sales (including MFP units) are submitted by May 30, 2026. CMS publishes each quarter's Part B ASP and payment limit files with a two-quarter lag (so Q1 data inform July 2026 pricing). Once MFP applies to payment, those payment-limit files will switch to showing 106% MFP for selected drugs ¹ ².

Part B Payment Rules and Transitions

- **Before MFP applicability:** Traditionally, Medicare Part B pays **106% of ASP** for single-source drugs (42 U.S.C. 1395w-3a(b)(1)(B)). The ASP-based payment limit continues through the 2026–2027 negotiation period for any selected drugs that have Part B use (even if they have a Part D negotiated price), *unless* they are specifically renegotiated in cycle 2 ³.
- **Initial price applicability year (2026–2027):** The IRA's “initial price applicability year” is 2026–2027. For those years, CMS guidance states that **Part B payments remain ASP-based** for selected drugs that already have Part B utilization. Only if a drug is selected again in cycle 2 will the 2026–27 negotiated Part D price affect Part B payment ³. In short, **no payment change in 2026–2027** for Part B; ASP+6% remains the method.
- **Price applicability years starting 2028:** Beginning in 2028, for each selected drug, the statute directs CMS to set the Part B payment limit at *106% of the MFP* ². The Part B Payment Limit Overview confirms: “Beginning in...2028...the payment limit...during the price applicability period [is] 106 percent of the MFP.” ² At that point, CMS stops updating ASP-based payment limits for that drug; instead the payment file will list the 106% MFP rate. (CMS notes in the PFS final rule FAQ that in quarters “in which Medicare payment is based on MFP, the...Payment Limit File will display the MFP-based payment limit” ¹.)
- **Beneficiary Liability:** Patients' coinsurance is 20% of the payment limit. So when payment switches to 106% MFP, beneficiary coinsurance shifts to 20% of (106% MFP), rather than 20% of (106% ASP). Given that negotiated MFPs are typically set below existing prices, providers may see reduced payments on those drugs after 2028, and beneficiaries lower coinsurance dollars as well.

Implications and Downstream Effects

- **Effect on ASP levels:** Including MFP units in ASP calculation will generally *raise* the reported **ASP** for selected drugs *relative to the scenario where they are excluded*. (This is because ASP is a volume-weighted average: adding lower-priced sales normally *lowers* the average. However, removing the lower-priced MFP sales entirely from the calculation *removes a downward weight*,

artificially inflating ASP. CMS's inclusion rule avoids that inflation.) In effect, reported ASP may be slightly higher (closer to the pre-MFP level) than it would have been if CMS had excluded those units.

- **Rebates and Best Price:** MFP rebates are not Medicaid rebates (they are payments to Medicare, not state Medicaid). CMS has not indicated that MFP prices become the “best price” in Medicaid or influence other drug-price programs. Typically, “best price” excludes prices only when certain exclusions apply (e.g., nominal sales, government programs, etc.), and a Medicare-negotiated rebate likely isn’t counted. However, if a company misinterprets MFP rebates as best-price-type sales, that could trigger Medicaid rebate liabilities or even civil monetary penalties. Manufacturers should consult legal counsel and possibly confirm with HHS OIG/CMS if needed.
- **340B Program:** Under current rules, 340B purchasers pay the 340B ceiling price (often calculated as ASP minus 22.5%). If ASP changes due to MFP, it could nominally alter the ceiling price. CMS’s PFS rulemaking did not modify 340B ASP exclusions; 340B purchases remain included in ASP (as before) unless a specific exclusion is added. However, any shift in ASP will indirectly change 340B calculations. (Separately, CMS’s Inflation Rebate PFS policies address 340B rebates under Part D, but that is outside Part B ASP.)
- **Part D and Commercial Plans:** Medicare Part D plan reimbursements are generally independent of Part B ASP. However, Part D plans may use an underlying ASP for certain calculations (e.g., 340B adjustments, or in formulary designs). Commercial insurers and Medicare Advantage providers sometimes use ASP or average wholesale price (AWP) benchmarks to pay for physician-administered drugs. Any change in reported ASP could affect these contracts. Manufacturers should assess whether contracts tie reimbursements or triggers to ASP relative to WAC or best price.
- **Compliance Risk:** CMS will treat failure to include MFP sales as an ASP-reporting error. The PFS final rule made clear this is a policy change (effective 2026) and that compliance begins then ¹. Companies that erroneously exclude MFP units risk audit adjustments or sanctions under existing ASP reporting rules (42 CFR §414.904). Likewise, companies should be careful in applying discounts or “rebates” in reporting – a common error would be to incorrectly exclude the MFP rebate as a Medicaid rebate or other exempt item. The safest approach is to net the sales to the MFP, report them, and clearly label any rebate transactions in financial records.
- **Budgetary Impact:** While outside manufacturer control, it’s worth noting that raising ASP (vs excluding MFP) means Medicare continues paying roughly ASP+6% on those drugs until 2028, rather than switching earlier. CMS’s intent was to avoid double-counting savings: if they excluded low MFP sales from ASP, ASP+6% would drop, then switching to 106% of MFP would save twice. By including them, CMS ensures the system shifts to MFP payment only once, per law ¹ ².

Open Questions & Recommendations

- **Treatment of Rebates/Retroactive Adjustments:** The guidance assumes “sales at MFP” directly. It is less clear how to report if a manufacturer sells at higher WAC and then must **rebate down to MFP**. One interpretation: report the gross sale in the quarter it occurred and then in the same (or subsequent) quarter report a price concession equal to the rebate. Another: net the sale at MFP immediately, so that only the net remains. CMS has not published detailed rules on this nuance. We

recommend documenting either approach and being prepared to explain it. (Given CMS's emphasis on including units, a netting approach may better align with their intent.)

- **Quarterly vs Annual Reconciliation:** If manufacturers accumulate MFP refunds and only pay them retroactively, the timing of reporting may not match sales. Companies should develop internal processes to attribute rebates to the correct quarter's ASP report. As ASP reporting allows estimates (a 12-month rolling adjustment for lagged rebates ⁹), one may use an estimate in initial quarters and adjust later.
- **Medicaid Best Price Ambiguity:** Since the law and CMS guidance did not explicitly exclude negotiation rebates from Best Price, manufacturers should assume *best price is unaffected by the negotiation program*. Negotiated refunds are a payment from manufacturer to Medicare (via the MTF) and are not discounts offered to states or covered entities. However, they should monitor any future CMS guidance or OIG opinions on this question.
- **Monitor Rulemakings:** CMS indicated that implementation details will evolve. They are required to issue program instructions each year through 2028 ¹⁰. CMS also noted intentions to propose Part D rules related to negotiation under its Medicare authority (which could indirectly affect ASP for drugs common to Part D and B). Manufacturers should watch the Federal Register and HPMS for updates, and consider providing comments if CMS proposes clarifications (especially in ASP reporting FAQs or PFS rules).
- **Documentation and Audit Readiness:** CMS now expects supporting documentation for ASP assumptions. Keep detailed records of how MFP transactions are handled and include that rationale in the quarterly submission's explanatory letter. Even though CMS did not create a new form, any Certified ASP submission should footnote or comment on the inclusion of MFP sales. If CMS later audits ASP, being transparent will reduce compliance risk.

Timeline of Key Events

Date	Event
2022	Inflation Reduction Act signed; establishes Medicare Drug Price Negotiation Program (PHS Act §§1191–1198).
Oct 31, 2025	CMS issues CY 2026 PFS Final Rule: <i>"include units sold at MFP in ASP"</i> (effective Jan 1, 2026) ¹ .
2026–2027	First negotiation prices take effect (for selected drugs in Part D). For any selected drug also used under Part B, payment remains 106% ASP during these years ³ . Manufacturers include MFP sales in ASP reporting.
2028	First year where Part B payment switches to 106% MFP for selected drugs ² . ASP reporting still required, but ASP no longer determines Medicare payment for those drugs.

Sources

We relied on primary CMS materials and relevant law:

- **CMS Newsroom Fact Sheets and Overviews:** CY 2026 PFS Proposed/Final Rule fact sheets on ASP (Oct 2025, Mar 2026) ¹ ⁶ ; Part B Payment Limits Overview (Mar 2026) ² .
- **Statute (Social Security Act):** 42 U.S.C. §1395w-3a (ASP payment methodology, especially subsection (c) for definition) ⁴ ⁸ .
- **CMS Guidance Documents:** Medicare Part B Drug Payment Limits Overview (Mar 2026) for MFP payment transition ² ; ASP Quarterly Reporting FAQs for general methodology (Mar 2026) ¹¹ ¹² .
- **Industry Commentaries:** Covington LLP alert (Nov 2025) and JD Supra/other analyses summarizing CMS's MFP policy ¹³ ¹⁴ . (These helped interpret CMS language but all policy quotes are from CMS sources above.)

These sources provide the authoritative CMS language and dates regarding ASP/MFP treatment, and informed the analysis above.

¹ Calendar Year (CY) 2026 Medicare Physician Fee Schedule Final Rule (CMS-1832-F) | CMS
<https://www.cms.gov/newsroom/fact-sheets/calendar-year-cy-2026-medicare-physician-fee-schedule-final-rule-cms-1832-f>

² ³ Part B Payment Limit Overview
<https://www.cms.gov/files/document/part-b-drug-payment-limits-overview.pdf>

⁴ ⁷ ⁸ ⁹ Social Security Act §1847A
https://www.ssa.gov/OP_Home/ssact/title18/1847A.htm

⁵ ¹⁴ ASP reporting gets a major update in the CY 2026 PFS | McDermott Will & Schulte - JDSupra
<https://www.jdsupra.com/legalnews/asp-reporting-gets-a-major-update-in-3492786/>

⁶ Calendar Year (CY) 2026 Medicare Physician Fee Schedule (PFS) Proposed Rule (CMS-1832-P) | CMS
<https://www.cms.gov/newsroom/fact-sheets/calendar-year-cy-2026-medicare-physician-fee-schedule-pfs-proposed-rule-cms-1832-p>

¹⁰ Medicare Drug Price Negotiation Program: Final Guidance, Implementation of Sections 1191 – 1198 of the Social Security Act for Initial Price Applicability Year 2027 and Manufacturer Effectuation of the Maximum Fair Price in 2026 and 2027
<https://www.cms.gov/files/document/medicare-drug-price-negotiation-final-guidance-ipay-2027-and-manufacturer-effectuation-mfp-2026-2027.pdf>

¹¹ ¹² Part B Drug Technical Support (DTS) Average Sales Price (ASP) Quarterly Publication Process Frequently Asked Questions (FAQs)
<https://www.cms.gov/files/document/frequently-asked-questions-faqs-asp-data-collection.pdf>

¹³ CMS Implements Significant Policy Changes for Drug Manufacturers in CY 2026 Physician Fee Schedule Final Rule | Covington & Burling LLP
<https://www.cov.com/en/news-and-insights/insights/2025/11/cms-implements-significant-policy-changes-for-drug-manufacturers-in-cy-2026-physician-fee-schedule-final-rule>